جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : سحر احمد مجاهد احمد
المحافظة : مرسي مطروح **الحي / المدينة :** اخري **العنوان :** الريفيه
رقم البطاقة : 26611040101681 **رقم التليفون :** 01228922879 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 04-11-1966 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سحر احمد مجاهد احمد
National ID: 26611040101681

GUSTAVE ROUSSY INTERNATIONAL EGYPT

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Zoledronic acid for bone protection during hormonal therapy**

التاريخ : **05-07-2026**	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

[[DOCTYPE:CT_SCAN]]
CT Scan

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
امانة المراكز الطبية المتخصصة
مستشفى اورام دار السلام
ادارة الاشعة

Patient Name: Sahar Ahmed Mogahed Ahmed	Date : 28/05/2024
Exam : C.T ABDOMEN &PELVIS	Age : 57 Year(s)
Medical ID: 1643/2024	Pat. ID: 53780

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Total DLP=625

1. ***Chest:***

- Left breast UOQ irregular speculated mass lesion measuring 5 x3 cm with left axillary level I prominent lymph nodes the largest measures 0.85 cm.
- solitary right upper lobe, anterior segment tiny calcified pulmonary nodule less than 3 mm
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. ***Abdomen and pelvis:***

- The liver is average in size with homogenous CT texture. No definite focal lesions or biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. ***The scanned skeleton:***

- Spondylo-degenerative changes .
- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

Name: سحر احمد مجاهد احمد	
MR. : 776	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient was diagnosed with left-breast invasive ductal carcinoma, clinical stage T3 N1. Initial management included neoadjuvant chemotherapy (NAC) followed by left modified radical mastectomy (MRM) performed on 18/12/2026. Post-operative pathology confirmed invasive ductal carcinoma, grade 3, with estrogen receptor (ER) 8/8 positive, progesterone receptor (PR) negative, and HER2 negative. The patient subsequently commenced adjuvant therapy with an aromatase inhibitor (Arimidex). A bone scan demonstrated focal uptake in the left acetabulum and right femur. After consultation with Dr. Kelany, magnetic resonance imaging (MRI) of the hips was ordered to further evaluate these findings, and bone health management with calcium supplementation was initiated.

Pending investigations include MRI of the shoulder and MRI of the left hip. A computed tomography (CT) of the chest, abdomen, and pelvis performed on 12/2025 was reported as free of disease. The patient continues on Arimidex and calcium supplementation (1 tablet daily, 90 mg elemental calcium). An additional bone-protective agent (One Alpha) was prescribed.

Follow-up after left-breast cancer surgery (post-mastectomy) confirmed the same pathology (IDC, grade 3, ER-positive, HER2-negative). The patient remains on adjuvant Arimidex. Bone scan performed in November 2025 showed foci of increased uptake in the right acetabulum and the trochanteric region of the left femur; MRI of both hip joints is pending to further characterize these lesions. The plan is to continue Arimidex for one month and to reassess imaging results when available.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : **ايه مدحت بدوي محمد**

يعتمد ,

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

Patient Name	: الأستاذه / سحر احمد مجاهد احمد	Request No.	: 13006499
Patient No.	: 26611040101681	Request Date	: 18/12/2024
Path No.	: 5068/24	Verification Date	: 01/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 58 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.

Nature Specimen: Left modified radical mastectomy specimen.

Gross Examination:
* Procedure: Left MRM measured 20x18x4 cm with covering skin ellipse measured 19x9 cm.
* Tumor extent: An ill defined greyish white firm tumor bed site.
* Size: 8.5 x8 x4 cm.
* Skin involvement: Nipple & areola were unremarkable grossly.
* Distance from margins: 0.1 cm away from deep margin & 0.2cm from the skin.
* Lymph nodes number and size: FFTP measured 6x3x2 cm and separate fibrofatty tissue piece in the same container measured 6x6x6 cm entangling 20 L.Ns up to 2x1 cm.

Microscopic Examination:
* Histological type: Invasive duct carcinoma with focal mucoid activity.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II .
- Maximal tumor diameter 8.5x8 cm
- Actual tumor cellularity: 80%, tumor necrosis index: 20%
* Ductal carcinoma in situ: major DCIS (non comedo ,intermediate nuclear grade).
* Lymphovascular emboli: Positive.
* Perineural invasion: Positive.
* Skin involvement: Free nipple & areola.
* Lymph node involvement: Poitive in (3/20) with no extracapsular extention. Maximal tumour diameter in lymph node 6 mm.
* Resection Margins: Free deep margin
* Histologic response to therapy: Partial pathological response.

Diagnosis:

Verified by :
Dr.Toka Medhat

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

Patient Name	: الأستاذه / سحر احمد مجاهد احمد	Request No.	: 13006499
Patient No.	: 26611040101681	Request Date	: 18/12/2024
Path No.	: 5068/24	Verification Date	: 01/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 58 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Left breast cancer, post NACT, MRM specimen:
- Invasive duct carcinoma GII with focal mucoid activity with major DCIS.
-Free deep margin.
- Free nipple & areola.
- Lymph nodes: Positive in (3/20) with no extracapsular extention
- Partial pathological response to therapy.
- Stage: ypT3, ypN1.
- RCB: 4.281, RCB class III.

Signature
Prof. Dr. Lubna Omar
Dr. Menna Nabil

Verified by :
Dr.Toka Medhat

CAP ACCREDITED
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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

Patient Name	الأستاذه / سحر احمد مجاهد احمد	Request No.	13008434
Patient No.	26611040101681	Request Date	15/05/2024
Path No.	1585/24	Verification Date	26/05/2024
Sex	Female	Financial Category	اورام دار السلام - مبادره
Age	57 Y	Department	Out Patient
Referreing Doc.			
		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense > 10% of tumor cells.

Signature
Prof. Dr. Gamal Abd-El-Atty.
Dr. Toka Medhat.

Verified by :
Dr.Duaa Mohamed

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[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

Patient Name	: الأستاذه / سحر احمد مجاهد احمد	Request No.	: 13008434
Patient No.	: 26611040101681	Request Date	: 15/05/2024
Path No.	: 1585/24	Verification Date	: 26/05/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 57 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast 2 O'clock solid mass lesion.

Nature of Specimen: Tru-cut biopsy, collected at 14/05/2024.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80 % of tumor cells (score 8/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Negative membrane reaction tumor cells (score 0).
- Ki67: 10 % nuclear reaction of tumor cells.

Conclusion:
Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II, With focal DCIS.
IPT:
- ER: Positive (score 8/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67: Positive in 10% of tumor cells.

* Scoring system: "ER, PR"

Verified by :
Dr.Duaa Mohamed

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[[DOCTYPE:MAMMOGRAM]]
Mammogram

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Sahar Ahmed Mogahed Ahmed	Date : 14/05/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 57 Year(s)
Medical ID: 26611040101681	Pat. ID: 53780

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

Left breast palpable mass

Report:

- ACR b: scattered fibroglandular parenchyma
- Left Breast:
  - Irregular dense mass is seen in the UOQ with overlying skin thickening, measuring 105x62x96mm.
  - Ultrasound shows this mass as irregular heterogeneous infiltrative mass extending from 12 to 3 O'clock, reaching the nipple & 3mm from overlying skin.
  - One indeterminate level I enlarged axillary lymph node showing focal cortical thickness (likely reactive)
  - No supra or infraclavicular lymph node.
- Right Breast:
  - No suspicious micro calcifications or architectural distortion detected.
  - Complementary ultrasound no cystic or solid mass lesions.
  - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
  - Nonspecific axillary LN.

Opinion:

- Locally advanced left breast malignant mass and level I indeterminate (likely reactive) axillary lymphadenopathy for histopathological assessment (BIRADS 5)
- Normal sonomammography of right breast (BIRADS 1)

Best regards,
Dr. Eman Abdel Razik, MSc, FEBR
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
14/05/2024 2:07 PM

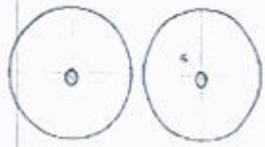
[[DOCTYPE:MDT_FORM]]
MDT Form

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : سحر احمد مجاهد احمد
Sex: Female
3- Medical number: 83209041
Old Med :
1643/2024
2-ID number:
Patient Age : 58 Y



Family history		Phone	01228922879	BMI		REASON OF MDT	
marital status		NRS Score		GROUP		Date of MDT	03/02/2025
Co morbidity		Menopausal status		Initial STAGE			

Clinical data

Breast Imaging :
LT 12-3 o'clock locally ADVANCED reaching nipple LN

Mets Workup :
CT CAP -free

BX	ER	PR	Her 2 Neu	SISH	Ki67
DCIS	8/8	0/8	0		10

T	N	M
3	IN DETERMINATE	0

Sonomammography

PATIENT HISTORY

receievd NACT taxotere x4-->AC x4-->sluggish response
MRM:free deep margin , nipple , areola YPT3 N1
LN 3/20
partial pathological response

MDT OUTCOME & PLAN

PORT
ADJUVANT AI
ZOMETA /6 MONTHS AFTER DENTAL EXAM

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	